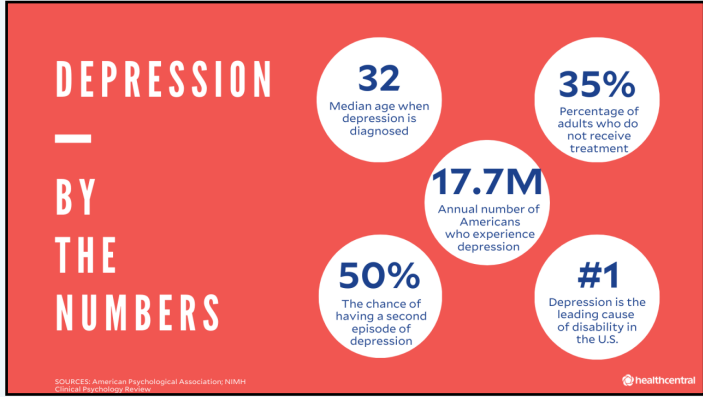




Managing Depression in Virtual Care

Why is This Important?



Based on the statistics, some of these patients will seek care through GCN, and we need the tools to manage and support them.

How to Assess

- Most important is to assess for acute safety risks (e.g., harm to self or others, psychotic features)
- Use the PHQ-9, or other validated tool, to quantitatively measure depression severity in initial treatment planning.
- The PHQ-9 takes approximately 2-5 minutes to administer. No formal training is required to use the measure.

PHQ-9				
Over the last 2 weeks, how often have you been bothered by any of the following problems? (Use "✓" to indicate your answer)	Not at all	Several days	More than half the days	Nearly every day
1. Little interest or pleasure in doing things	0	1	2	3
2. Feeling down, depressed, or hopeless	0	1	2	3
3. Trouble falling or staying asleep, or sleeping too much	0	1	2	3
4. Feeling tired or having little energy	0	1	2	3
5. Poor appetite or overeating	0	1	2	3
6. Feeling bad about yourself — or that you are a failure or have let yourself or your family down	0	1	2	3
7. Trouble concentrating on things, such as reading the newspaper or watching television	0	1	2	3
8. Moving or speaking so slowly that other people could have noticed? Or the opposite — being so fidgety or restless that you have been moving around a lot more than usual	0	1	2	3
9. Thoughts that you would be better off dead or of hurting yourself in some way	0	1	2	3

(For office coding: Total Score _____ = _____ + _____ + _____)

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

Not difficult at all	Somewhat difficult	Very difficult	Extremely difficult
☐	☐	☐	☐

How Do We Treat Depression?

What is behavioral health care?

Behavioral health care assesses, treats, and provides support for a variety of mental, emotional, and behavioral conditions.

At Kaiser Permanente, adults, families, teens, and children can receive clinical and support services for a range of behavioral health matters, including:

- ADHD
- Alcoholism/Addiction
- Anxiety
- Autism spectrum disorders
- Bipolar disorder
- Depression
- Eating disorders
- Obsessive-compulsive disorder
- Sleep problems
- Stress

You don't need a referral to see a therapist

You can make an appointment to see a therapist without a referral from your primary care physician. As a member, you have open access to most Kaiser Permanente Behavioral Health services.

Call 1-800-900-3277. Anytime.

You can call the Behavioral Health Care Help Line around the clock for direct and immediate access to crisis intervention and guidance, referrals, and resources.

For emergency care*

If you think you have a medical or psychiatric emergency, call 911 or go to the nearest hospital.

All Kaiser Permanente services are confidential. Services vary by region.

*If you reasonably believe you have an emergency medical condition, call 911 or go to the nearest Emergency Department. An emergency medical condition is a medical or psychiatric condition that requires immediate medical attention to prevent serious jeopardy to your health. For the complete definition of an emergency medical condition, please refer to your Evidence of Coverage.

- Psychotherapy can be used as first-line treatment or in addition to antidepressants.
- It has been shown to be as effective as antidepressants for patients with mild to moderate major depression.
- Discuss general measures such as lifestyle changes (healthy eating and exercise) and sleep hygiene.
- 1-800-900-3277 for **immediate** access to a licensed therapist (24/7).
- Call 1-833-579-4848 (M-F 8 a.m.—5 p.m.) to **schedule** an appointment with a BHT.

Pharmacotherapy

Ideally, medication and monitoring should be under the guidance of patient's primary care physician. However, we need to know how to manage questions or concerns for patients that reach out to us.

Some caveats to keep in mind:

- While the adverse effects of antidepressants may manifest earlier, their onset of action is at least 2 – 4 weeks.
- Sudden cessation of antidepressant medication may result in discontinuation symptoms; these may manifest as flu-like symptoms, insomnia, nausea, and hyperarousal.
- There is little difference in terms of efficacy between the various classes of antidepressants, but they do differ in terms of potential for drug interaction as well as side effects.

The table below is from the Clinical Guidelines and gives useful information about the different medication options.

Table 1: Medications Used in the Treatment of Depression						
Medication	Daily Dose Starting/ Usual ^a	Titration ^b	Max Daily Dose	Common Side Effects	Comments	Relative Cost ^c
- Bupropion, fluoxetine, paroxetine, duloxetine, and sertraline should be avoided in patients on tamoxifen therapy for breast cancer. Preferred alternatives include citalopram and venlafaxine. - Monitor patients for suicidal ideation or behavior. - Paroxetine should not be used in pregnancy. Caution with all other antidepressants in pregnancy.						
SSRIs						
fluoxetine (Prozac) ^a	10 mg / 20 - 40 mg	10 mg q 1 - 2 weeks	80 mg	- Insomnia, agitation, GI distress, sexual dysfunction, sedation. - Theoretical risk of serotonin syndrome if used with triptans. - Risk of GI bleed in elderly using NSAIDs. - May increase risk of bleeding with concurrent antiplatelet/anticoagulant drugs.	Avoid in breast feeding unless prior positive response.	1 (20 mg once daily)
citalopram (Celexa) ^a	20 mg / 20 - 40 mg	10 mg q 1 - 2 weeks	40 mg*		* 20 mg max daily dose for patients ≥ 60 years of age or concomitant cimetidine/ CYP2C19 inhibitors.	2 (20 mg once daily)
sertraline (Zoloft) ^a	50 mg / 50 - 150 mg	50 mg q 1 - 2 weeks	250 mg		Avoid in breast feeding unless prior positive response.	3 (100 mg once daily)
paroxetine (Paxil) ^a	10 mg / 20 - 40 mg (10 mg*)	10 mg q 1 - 2 weeks	80 mg		CAUTION in pregnancy.	4 (20 mg once daily)
SSNIs						
venlafaxine (Effexor) ^a	25 mg / 75 - 225 mg	75 mg q 1 - 2 weeks	300 mg	- Insomnia, agitation, GI distress (constipation and nausea), sexual dysfunction, sweating. - Theoretical risk of serotonin syndrome if used with triptans.		30 (75 mg BID)
venlafaxine XR capsules ^a	37.5 mg / 75 - 225 mg	75 mg q 1 - 2 weeks	300 mg			10 (150 mg once daily)
duloxetine (Cymbalta) NON-FORMULARY	20 mg / 60 mg	20 mg q 1 - 2 weeks	120 mg	May cause elevated blood pressure.		289 (60 mg once daily)
TCAs						
nortriptyline ^a	25 mg / 50 - 100 mg	25 - 50 mg q 1 - 2 weeks	150 mg	Dry mouth, constipation, urinary retention, drowsiness (may be more common in patients ≥ 65 years.)	Can cause cardiac arrhythmias and orthostatic hypotension	9 (75 mg HS)
desipramine ^a	25 mg / 100 - 200 mg	25 - 50 mg q 1 - 2 weeks	300 mg			260 (150mg HS)
OTHERS						
bupropion ^a (Wellbutrin)	75 mg BID / 150 - 300 mg	50 mg q 1 - 2 weeks	450 mg	Less sexual side effects, may increase agitation, may help with smoking cessation.	CAUTION in ETOH withdrawal and binge/purging patients. Contraindicated in patients with a history of seizures.	14 (100 mg TID)
bupropion SR ^a (Wellbutrin SR)	75 mg BID / 150 - 300 mg	50 mg q 1 - 2 weeks	450 mg			34 (2x 150 mg)
bupropion XL ^a (Wellbutrin XL)	75 mg BID / 150 - 300 mg	50 mg q 1 - 2 weeks	450 mg			15 (300 mg once daily)
mirtazapine ^a (Remeron)	15 mg / 30 - 45 mg	15 mg q 1 - 2 weeks	75 mg	Sedation.	Stimulates appetite, causes weight gain.	5 (30 mg once daily)
§ = Acquisition cost relative to fluoxetine = 1. G = generic available. † = Based on expert opinion. Δ = Lower starting doses are generally recommended for patients ≥ 65 years. * = Lower starting dose recommended in patients ≥ 65 years or with renal or hepatic compromise.						
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Introducing the GPC Mailbag!



At GPC Clinical Corner, we strive to provide tips on the best clinical practices, navigating patient encounters, and updated information on treatment options.

This year, we'd like to expand our scope to include your suggestions. We're proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you'd like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

HealthCentral. (2024). Depression: Everything you need to know. HealthCentral. <https://www.healthcentral.com/condition/depression>

Kaiser Permanente. (2023). Diagnosis and treatment of adult depression. Kaiser Permanente SCAL Clinical Library. <https://cl.kp.org/scal/cpg/cpg/html/mdd.html>